

PLEASE READ CAREFULLY

TOLCHICINE TABLET

Each **Tolchicine** tablet contains colchicine 0.6 mg.

DESCRIPTION

A white round biconvex tablet.

INDICATIONS

Tolchicine is indicated for:

- Treatment and prophylaxis of acute gouty attacks.
- Treatment and prophylaxis against recurrent attacks of pseudogout.

RECOMMENDED DOSAGE

Patients vary widely in their ability to tolerate the drug. Dosage should be varied according to bodyweight or individual reaction to colchicine toxicity.

- **Acute gout:** 1 - 2 tablets every 2 hours until the pain is relieved or until nausea, vomiting and diarrhoea occurs. The usual amount of colchicine required in a course of therapy may range from 4 - 8 mg. Pain and swelling usually abate within 12 hours and are usually gone within 48 - 72 hours.

Prophylaxis: Depending on the severity and patient's response and tolerance, the dosage varies from 1 tablet daily 1 - 4 times weekly to 1 tablet daily.

- **Pseudogout:** To relieve acute attack, 1 - 2 tablets every 2 hours until the pain is relieved or until nausea, vomiting and diarrhoea occurs.

Prophylaxis: Depending on the severity, patient's response and tolerance, up to 2 tablets daily.

Route of Administration: Oral

CONTRAINDICATIONS

- Hypersensitivity to the active substance or to any of the excipients listed.
- Patients with blood dyscrasias.
- Pregnancy and breastfeeding.
- Women of childbearing potential unless using effective contraceptive measures.
- Patients with severe renal and hepatic impairment.
- Patients undergoing haemodialysis since it cannot be removed by dialysis or exchanged transfusion.
- Patients with renal or hepatic impairment who are taking a P-glycoprotein (P-gp) inhibitor or a strong CYP3A4 inhibitor.

WARNINGS AND PRECAUTIONS

Tolchicine is potentially toxic so it is important not to exceed the dose prescribed. **Tolchicine** has a narrow therapeutic window. The administration should be discontinued if toxic symptoms such as nausea, vomiting, abdominal pain and diarrhoea occur. **Tolchicine** may cause severe bone marrow depression (agranulocytosis, aplastic anaemia, and thrombocytopenia). The change in blood count may be gradual or very sudden. Aplastic anaemia in particular has a high mortality rate. Periodic checks of the blood picture are essential.

If patients develop sign and symptoms that could indicate a blood cell dyscrasia, such as fever, stomatitis, sore throat, prolonged bleeding, bruising, or skin disorders, treatment

with **Tolchicine** should be immediately discontinued and a full haematological investigation should be conducted straight away.

Caution is advised in case of:

- 1) liver or renal impairment
- 2) cardiovascular disease
- 3) gastrointestinal disorders
- 4) elderly and debilitated patients
- 5) patients with abnormalities in blood counts

Co-administration with P-gp inhibitors and/or moderate or strong CYP3A4 inhibitors will increase the exposure to **Tolchicine**, which may lead to colchicine-induced toxicity including fatalities. If treatment with a P-gp inhibitor or a moderate or strong CYP3A4 inhibitor is required in patients with normal renal and hepatic function, a reduction in **Tolchicine** dosage or interruption of **Tolchicine** treatment is recommended.

Tolchicine contains lactose. Patients with rare hereditary problems of galactose intolerance, the Lapp lactase deficiency or glucose-galactose malabsorption should not take this medicine.

Tolchicine contains sodium metabisulphite that may cause serious allergic type reactions in certain susceptible patients. Do not use if known to be hypersensitive to bisulphites.

INTERACTIONS

Colchicine is a substrate for both CYP3A4 and transport protein P-glycoprotein. In the presence of CYP3A4 or P-glycoprotein inhibitors, the concentrations of colchicine in the blood increase.

Potential risk of severe drug interactions, including death, in certain patients treated with colchicine and concomitant P-glycoprotein or strong CYP3A4 inhibitors such as clarithromycin, ciclosporin, erythromycin, calcium channel blockers (verapamil and diltiazem), telithromycin, ketoconazole, itraconazole, voriconazole, HIV protease inhibitors, disulfiram and nefazodone.

P-glycoprotein or strong CYP3A4 inhibitors are not to be used in patients with renal or hepatic impairment who are taking colchicine.

A dose reduction or interruption of colchicine treatment should be considered in patients with normal renal and hepatic function if treatment with a P-glycoprotein or a strong CYP3A4 inhibitor is required. A 4-fold reduction of colchicine dosage is recommended when co-administered with a P-glycoprotein inhibitor and/or a strong CYP3A4 inhibitor. A 2-fold reduction in colchicine dosage is recommended when co-administered with a moderate CYP3A4 inhibitor.

In addition, substances such as cimetidine and tolbutamide reduce metabolism of colchicine and thus plasma levels of colchicine increase.

Grapefruit juice may increase plasma levels of colchicine. Avoid consuming grapefruit and grapefruit juice while using colchicine.

Reversible malabsorption of cyanocobalamin (vitamin B12) may be induced by an altered function of the intestinal mucosa.

The risk of myopathy and rhabdomyolysis is increased by a combination of colchicine with statins, fibrates, ciclosporin or digoxin.

PREGNANCY AND LACTATION

Pregnancy – colchicine is genotoxic *in vitro* and *in vivo*, and is teratogenic in animal studies. **Tolchicine** is therefore contraindicated in pregnancy. Women of childbearing potential have to use effective contraception during treatment.

Breastfeeding – colchicine is excreted in breast milk. Therefore, use of **Tolchicine** is contraindicated in women who are breastfeeding.

Fertility – colchicine administration in animals induces significant reductions in fertility.

SIDE EFFECTS

Blood and lymphatic system disorders

Not known: Bone marrow depression with agranulocytosis, aplastic anaemia and thrombocytopenia.

Nervous system disorders

Not known: Peripheral neuritis, neuropathy.

Gastrointestinal system disorders

Common: Abdominal pain, nausea, vomiting, and diarrhoea. Not known: Gastrointestinal haemorrhage.

Hepatobiliary disorders

Not known: Hepatic damage.

Skin and subcutaneous tissue disorders

Not known: Alopecia, rash.

Musculoskeletal and connective tissue disorders

Not known: Myopathy and rhabdomyolysis.

Renal and urinary disorders

Not known: Renal damage.

Reproductive system and breast disorders

Not known: Amenorrhoea, dysmenorrhoea, oligospermia, azoospermia.

Others

Angioneurotic oedema, epistaxis, chromosomal dysfunction.

SYMPTOMS AND TREATMENT OF OVERDOSE

Tolchicine has a narrow therapeutic window and is extremely toxic in overdose. Patients at particular risk of toxicity are those with renal or hepatic impairment, gastrointestinal or cardiac disease and patients at extremes of age.

Symptoms may be delayed (3 hours on average): nausea, vomiting, abdominal pain, haemorrhagic gastroenteritis, volume depletion, electrolyte abnormalities, leukocytosis, and hypotension in severe cases. The second phase with life-threatening complications develops 24 to 72 hours after drug administration: multisystem organ dysfunction, acute renal function, and confusion, coma, ascending peripheral motor and sensory neuropathy, myocardial depression, pancytopenia, dysrhythmias, respiratory failure, and consumption coagulopathy. Death is usually a result of respiratory depression and cardiovascular collapse. If the patient survives, recovery may be accompanied by rebound leukocytosis and reversible alopecia starting about one week after the initial ingestion.

Treatment: No antidote available.

- Elimination of toxins by gastric lavage within one hour of acute poisoning.
- Consider oral activated charcoal in adults who have ingested more than 0.1 mg/kg bodyweight within 1 hour of presentation and in children who have ingested any amount within 1 hour of presentation.

- Haemodialysis has no efficacy.
- Close clinical and biological monitoring in hospital environment.
- Symptomatic and supportive treatment: control of respiration, maintenance of blood pressure and circulation, correction of fluid and electrolyte imbalance.
- The lethal dose varies widely (7 – 65 mg single dose) for adults but is generally about 20 mg.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINES

No details are available regarding the influence of **Tolchicine** on the ability to drive and use machines. However, the possibility of drowsiness and dizziness should be taken into account.

PHARMACODYNAMICS

Gouty inflammation of the tissues or joint is associated with the local accumulation of urate microcrystals by phagocytic neutrophils. After sufficient amounts of the crystals have been taken up into the phagolysosomes of the neutrophils, these organelles disrupt and release their degradative enzymes, accumulated microcrystals and chemotactic factors. The released urate crystals restart the cycle, and the chemotactic factors, which are released together, will attract more neutrophils. These related substances are responsible for much of the local inflammation and pain associated with acute gout attack.

PHARMACOKINETICS

Colchicine does not alter the metabolism or excretion of uric acid and clinical situation other than inflammations associated with gout or pseudogout (calcium pyrophosphate deposition disease). It is thought that colchicine acts by preventing phagocytosis and the release of chemotactic factors, and this in turn decreases the attraction of more neutrophils into the affected area. Colchicine also binds to leukocyte microtubules in a reversible covalent complex and causes their depolymerization and may be involved in decreasing the attraction of the motile leukocytes into the inflamed area. By these mechanisms, this cycle: Leukocytic microcrystal phagocytosis → degradative product release → inflammation → leukocytic attraction → phagocytosis of microcrystals, is disrupted.

EXCIPIENTS

Sodium metabisulphite, magnesium stearate, sodium starch glycolate, povidone, talcum, lactose, corn starch.

PRESENTATIONS

PVC-Aluminium blister strip of 10 tablets per strip.

Blister pack of 100 x 10 tablets.

Store in a cool dry place, below 30 °C. Protect from light.

Keep out of reach of children.

Jauhi daripada kanak-kanak.

Shelf life: Please refer to outer box label.

Manufactured by:

T.O. Chemicals (1979) Ltd.

53 Lieb-Klong 7 Road, Moo 4,
Bueng Kham Phroi, Lam Luk Ka,
Pathumthani 12150, Thailand.

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